

State of Kenya's Health System 2026

Executive Summary

Kenya's health system enters 2026 mid-transition: a decade-old devolved structure carrying wide, persistent county disparities, layered onto a financing reform — the shift from the National Hospital Insurance Fund (NHIF) to the Social Health Authority (SHA) — that is structurally sound in design but under acute operational stress. This brief synthesises current national performance, county-level variation, and how Kenya's position compares internationally, to identify where strategic investment would do the most good.

1. National Performance Snapshot

Kenya's core health outcomes have improved steadily but remain below both national targets and regional peers on several measures.

Indicator	Value
Life expectancy at birth	66.7 years
Maternal mortality ratio	342 per 100,000 live births
Under-five mortality rate	42 per 1,000 live births
Skilled birth attendance	89% (up from 66% in 2014)
Immunisation coverage (24 months)	80%
Health workforce density (doctors, nurses, clinical officers)	30.1 per 10,000 population — 68% of the WHO SDG threshold (44.5)
Government health spend	≈4.5% of GDP; Ksh138.1 billion FY2025/26 budget (+8.7% y/y)

Sources: KNBS 2021; Kenya MoH Health Sector Report (National Treasury MTEF); WHO/task-sharing policy literature (PMC9336004); Willow Health Media, June 2025.

2. The SHA Transition: Progress and Strain

Kenya completed its shift from NHIF to the Social Health Authority (SHA) under the Taifa Care policy, launched October 2024. Registration has scaled quickly, but conversion to functioning coverage is lagging:

- Over 19.3 million Kenyans registered with SHA; only ~3.33 million had completed means-testing to determine premiums.
- Roughly 56% of active health facilities (8,813 of 17,755) are enrolled in SHA; about 89% of those have successfully transacted through the system.
- SHA has collected Ksh 81 billion since October 2024 but owed providers Ksh 30 billion in pending bills as of November 2025 — a liability inherited largely from NHIF.
- SHA registration coverage remains lowest in Turkana, West Pokot, Garissa and Samburu — the same counties that already carry the weakest health workforce and infrastructure.

The financing architecture is credible; the execution gap — claims processing, provider trust, and means-testing throughput — is the binding constraint on UHC progress through 2026.

Sources: Daily Nation, Jan 2026; Kenya Parliament Health Committee roundtable, Nov 2025; IEA Kenya SHA assessment, Mar 2026; RShots SHA review, Feb 2026.

3. County Comparisons: A Persistent Two-Tier System

Devolution has not closed Kenya's geographic health divide — in several respects it has made the variation easier to measure and harder to ignore.

- Workforce concentration: roughly half of the country's specialist physicians work in Nairobi, Mombasa and Kisumu alone; Turkana has approximately 1 doctor per 70,000 residents against a WHO benchmark of 23 doctors, nurses and midwives per 10,000.
- Clinical officer density is lowest in Turkana, Mandera, Garissa, Wajir and Tharaka Nithi — the same arid and semi-arid counties that face the longest travel distances to care.
- Facility access mirrors the same pattern: only 58% of the population lives within 20km of specialist services such as dental care, with access concentrated in six urbanised counties.
- Historical efficiency studies show county health systems operating at roughly 40% average hospital bed occupancy and around 53% development-budget execution — indicating the bottleneck is as much about absorption and management capacity as raw funding.

The strategic implication: allocation formulas that distribute funding by population alone under-serve counties with the least capacity to execute it. Investment strategy needs a capacity-weighted, not just needs-weighted, allocation logic.

Sources: IntraHealth, Turkana workforce case study; PMC7613432 county efficiency analysis; PMC9422806 Kenya Health Labour Market Analysis 2020; Annals of Global Health geospatial dental-access study.

4. International Benchmarking

Kenya's UHC trajectory sits between two more mature reference models in the region — useful both as caution and as a template.

- Rwanda: mandatory community-based health insurance (Mutuelles de Santé) rolled out from 2000, achieving near-universal enrolment and the lowest under-five and maternal mortality ratios among comparator countries — but built on two more decades of sustained implementation than Kenya has had.
- Ghana: expanded its National Health Insurance Scheme from district to national level in 2003, with the highest GDP per capita among comparator countries easing (not solving) financing pressure.
- Common finding across Ghana, Rwanda, Tanzania, Kenya and Ethiopia: social health insurance schemes have struggled to enrol informal-sector and indigent populations at scale, and fragmented risk pools have undermined cross-subsidy — a structural risk SHA now shares.

Kenya is not behind these peers in policy design; it is earlier in execution maturity, with the same enrolment and provider-payment risks Rwanda and Ghana faced — and largely solved — a decade or more ago.

Sources: Fenny, Yates & Thompson, Global Health Action (2021), comparative five-country SHI financing analysis; PMC10536813 Kenya–Rwanda digital financial services for health case study.

5. Strategic Investment Priorities

Three priorities emerge directly from the evidence above, in order of where marginal investment currently has the highest expected return:

- Provider payment infrastructure. Clearing the Ksh 30 billion SHA pending-bills backlog and hardening claims-processing systems is the single fastest way to restore provider participation and protect the coverage already achieved.
- Capacity-weighted workforce deployment. Target the ~10 lowest-density counties (Turkana, Mandera, Garissa, Wajir, Tharaka Nithi, West Pokot, Samburu) with retention-linked incentives, not just facility construction — the evidence points to retention and management capacity, not infrastructure alone, as the binding constraint.
- Means-testing throughput. Closing the gap between 19.3 million SHA registrants and 3.33 million means-tested individuals is a prerequisite for both equitable premium-setting and accurate national coverage reporting — without it, Kenya cannot reliably measure its own progress toward UHC.

6. What This Brief Does Not Yet Cover

This is the Executive layer of Flagship Project 01. It intentionally does not include county-by-county quantitative modelling, international benchmarking against a wider country set, or investment costing — those sit in the Analytics and further Thought Leadership layers of this project, to be developed next.